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ROLE OF HR POLICIES IN PROMOTING EMPLOYEE WELLBEING AND MENTAL HEALTH

Jeenal Gautam

MBA IV, Kalinga University, Raipur, India

ABSTRACT:

The mental health and overall wellbeing of employees have emerged as one of the most critical priorities in contemporary human resource management (Guest, 2017; World Health Organization [WHO], 2022). In an era distinguished by increasing workplace stress, economic uncertainty, and blurring boundaries among professional and personal life, the role of HR policies in promoting employee wellbeing and mental health has never been more important (Cooper & Cartwright, 1994; International Labour Organization [ILO], 2022). This research paper studies how HR policies contribute to employee wellbeing, psychological health support, and work-life balance in Indian organizations (Kumar & Kumar, 2021).

The study adopts a descriptive research design using primary data collected through a structured Google Forms questionnaire distributed among 62 respondents including full-time employees, part-time workers, and professionals among various age groups and genders. The findings disclose important findings: 84% of respondents experience workplace stress at least sometimes, 49% lack access to psychological health support, and 82% have taken stress-related leave at least occasionally. Although these problems exist, 87% agreed that HR policies help reduce workplace stress, 90% believe mental health policies impact employee retention, and 91% support mandatory mental health training for managers (Deloitte, 2023; CIPD, 2023).

The paper concludes that while HR policies have achieved considerable progress in promoting employee wellbeing, major gaps remain in psychological health support availability, policy accessibility, and leadership commitment (WHO, 2022). Organizations must urgently commit to comprehensive mental health programs, flexible working arrangements, and manager training to build workplaces that sincerely support employee psychological health and organizational sustainability (Grawitch et al., 2006; ILO, 2022).

Keywords: HR Policies, Employee Wellbeing, Mental Health, Work-Life Balance, Workplace Stress, Organizational Commitment

INTRODUCTION

1.1 Background of the Study

The relationship between corporate policies and employee health has undergone a fundamental change over the past two decades. Traditionally, Human Resource Management focused primarily on managerial aspects such as recruitment, performance appraisal, and compensation (Jyothi & Venkatesh, 2013). Today, however, there is a rising recognition that the psychological wellbeing and mental health of employees are equally key determinants of organizational performance, productivity, and sustainability (Guest, 2017; Kaur, 2023, pp. 176–181; Peretz, 2024).

Employee wellbeing refers to the overall quality of an individual's experience at work, covering physical health, mental health, social relationships, and a feeling of purpose and fulfilment (Grant et al., 2007; Inc., 2023). Mental health, as defined by the World Health Organization, is a state of wellbeing in which an individual realizes their own potential, can cope with normal life stresses, can work productively, and is able to contribute to their community (World Health Organization [WHO], 2022). When mental health is compromised in the workplace through excessive stress, burnout, lack of support, or poor work-life balance, the consequences are serious, affecting not only the individual employee but also team performance, organizational culture, and business outcomes (Maslach & Leiter, 2016; Muneer, 2024, p. 33).

HR policies serve as the organizational framework through which companies operationalize their devotion to employee wellbeing. These policies govern working hours, leave entitlements, flexible arrangements, mental health support programs, employee assistance programs, and leadership accountability for wellbeing outcomes (Armstrong & Taylor, 2020). The design, accessibility, and implementation of these policies immediately determine whether employees experience their workplace as a source of support or a source of stress (Bhoir & Sinha, 2024; Right to Disconnect Bill, 2025 (India), 2025).

1.2 Research Problem

Despite growing organizational awareness of employee wellbeing, many organizations persist in struggling to implement HR policies that successfully address mental health and stress-related challenges. Employees

frequently report limited access to mental health support, insufficient flexible working arrangements, and inadequate leadership training on wellbeing issues (Shekhar & Saprobe, 2025). The difference between policy intention and practical experience is still significant, particularly for young and part-time employees who may be less visible to HR and management teams (Jain, 2025, pp. 1–10). This study examines the real-world effectiveness of HR wellbeing policies from the employee perspective (Guest, 2017).

1.3 Objectives of the Study

The objectives of this research are as follows:

- To examine employee awareness of and access to HR policies related to wellbeing and mental health.
- To assess the prevalence of workplace stress and mental health issues among employees.
- To evaluate the effectiveness of HR policies in promoting work-life balance and reducing workplace stress.
- To understand employee perceptions of leadership support for wellbeing initiatives.
- To identify the most important factors for employee wellbeing from the employee perspective.
- To provide recommendations for improving HR policies related to employee wellbeing and mental health.

1.4 Scope and Significance of the Study

This study is significant for several reasons. First, it offers empirical evidence from the employee perspective on the effectiveness of HR wellbeing policies, a viewpoint that is frequently underrepresented in organizational research (Grawitch et al., 2006). Second, it illustrates the vital need for organizations to address mental health as a strategic HR priority rather than a peripheral concern (WHO, 2022). Third, it improves the growing academic literature on employee wellbeing in the Indian work setting, where fast economic development and changing work patterns have created unique wellbeing challenges (Ganuthula & Balaraman, 2025; S, 2025).

The study concentrates on employees among different employment types, age groups, and genders, with particular relevance to young professionals between 20 and 30 years of age who represent an increasingly important segment of the Indian workforce (Thareja, 2025, pp. 8–18).

LITERATURE REVIEW WITH RESEARCH GAP

The academic literature on employee wellbeing and HR policies is extensive and spans multiple disciplines including organizational psychology, human resource management, public health, and organizational behaviour (Bhoir & Sinha, 2024; Guest, 2017). Several foundational conceptual models and empirical studies provide important context for understanding the role of HR policies in promoting employee wellbeing.

Herzberg's Two-Factor Theory identified hygiene factors—including working conditions, company policies, and supervision—as fundamental prerequisites for employee satisfaction. When these factors are inadequate, dissatisfaction and stress result (Herzberg et al., 1959). The presence of strong HR wellbeing policies can therefore be understood as a hygiene factor that prevents workplace distress and enables personnel to focus on more intrinsically motivating aspects of their work (Herzberg's Two-Factor Theory: Distinguishing Motivators and Hygienic Factors, 2024).

Karasek's Job Demands-Control Model proposed that high job demands combined with low job control create the highest levels of workplace stress and health risks (Karasek, 1979). This model directly supports the significance of flexible HR policies that give employees greater control over their working arrangements, consequently reducing the psychological burden of demanding work environments (Matthews & Ritter, 2015).

Research by the World Health Organization estimated that depression and anxiety cost the global economy approximately one trillion US dollars annually in lost productivity (World Health Organization [WHO], 2022). This figure illustrates the enormous economic case for investing in workplace mental health programs, beyond the obvious humanitarian imperative. More recent global estimates also indicate that over one billion people live with mental health conditions, highlighting the urgency of large-scale intervention (Over a Billion People Living With Mental Health Conditions – Services Require Urgent Scale-Up, 2025).

Studies on Employee Assistance Programs (EAPs) have consistently shown that organizations supplying comprehensive mental health support have lower absenteeism, higher productivity, and improved employee retention rates (Mushtaq, 2025, pp. 1–5; Zieringer & Zapf, 2024, pp. 280–298). Baicker, Cutler, and Song (2010) found that wellness programs yield an average return of approximately \$3.27 for every dollar invested, largely through diminished healthcare costs and absenteeism.

Research regarding work-life balance has established that flexible working arrangements are among the most effective organizational interventions for reducing workplace stress and improving worker wellbeing (Impact of Job Stress and Work-Life Balance on Turnover Intention Among Indian Healthcare Workers, 2026).

Greenhaus and Allen (2011) found that work-family balance positively predicts life satisfaction, job satisfaction, and overall health outcomes, supporting the strategic significance of flexible HR policies.

Despite the richness of global literature, a major research gap remains in the Indian organizational context. Much of the existing research focuses on multinational corporations or Western workplaces, while limited empirical evidence is available regarding employee perceptions of HR wellbeing policies in Indian organizations, especially among young professionals, part-time employees, and workers in hybrid employment arrangements (Kaur, 2023; Jain, 2025). Additionally, many studies emphasize employer-reported initiatives rather than employee-experienced effectiveness. Therefore, this study seeks to bridge this gap by examining how employees themselves perceive the accessibility, usefulness, and impact of HR wellbeing policies in India.

Research Gap

While the literature on employee wellbeing and HR policies is rich at the international level, there continues to be a significant gap in empirical research examining the specific experiences of Indian employees—particularly young, part-time, and early-career workers—with respect to HR health and wellbeing policy effectiveness and mental health support (Agnihotri et al., 2024; Guest, 2017). Most existing Indian studies focus on large corporations or specific sectors, leaving a gap in understanding how wellbeing policies are experienced across varied organizational types and employment arrangements (Patel, 2025; Kaur, 2023).

Furthermore, much of the available literature emphasizes organizational policy design rather than employee perceptions of accessibility, implementation, and actual usefulness of such policies (Grawitch et al., 2006). Limited research has explored whether employees are aware of these policies, trust them enough to use them, or perceive them as meaningful support systems in managing stress and work-life balance (World Health Organization [WHO], 2022).

This study intends to address this gap by providing direct employee-level insights into HR policy effectiveness from the Indian organizational context, with a focus on diverse employment categories and contemporary workplace wellbeing challenges.

DATA COLLECTION AND RESEARCH METHODOLOGY

3.1 Research Design

This study adopts a descriptive research design to examine employee perceptions of HR policies related to wellbeing and mental health. Descriptive research is appropriate here because it permits the systematic gathering and examination of data about current employee experiences and organizational practices without manipulating variables (Kothari, 2004; Baiju, 2025). The goal is to describe the nature and effectiveness of HR wellbeing policies as experienced by real employees through different organizational contexts (Saunders et al., 2019).

3.2 Research Approach

A quantitative research approach was used, involving the collection of numerical data through a standardized questionnaire. This approach enables the measurement of employee perceptions, attitudes, and experiences in a standardized, comparable format (Creswell & Creswell, 2018). Statistical analysis was then applied to discover patterns and obtain meaningful conclusions about HR health policy effectiveness (Singh et al., 2025).

3.3 Data Sources

Primary Data: Primary data was collected through a structured Google Forms questionnaire distributed among 62 respondents, including full-time employees, part-time workers, and professionals throughout various age groups and genders. Questions covered HR policy awareness, mental health support availability, workplace stress levels, work-life balance perceptions, and leadership support for wellbeing.

Secondary Data: Secondary data was gathered from academic journals, HR management textbooks, World Health Organization reports, and previously published research on employee wellbeing, workplace mental health, and organizational HR practices (World Health Organization [WHO], 2022; Armstrong & Taylor, 2020).

3.4 Sampling Technique

Convenience sampling was used to select respondents. This method was chosen for its usefulness as well as efficiency in collecting data from accessible and willing participants (Etikan et al., 2016). While this limit the

generalizability of findings, it is appropriate for an exploratory study of this nature. Future research with larger and more varied organizational samples would help validate and broaden these outcomes (Bryman & Bell, 2015).

3.5 Sample Size

A total of 62 respondents participated in the study. The sample included employees from diverse backgrounds, with a predominant representation of young adults aged 20–25 (77%) and a near-equal split between full-time (44%) and part-time (56%) employees. This demographic profile is particularly relevant because younger workers represent an increasingly important segment of the modern workforce (Thareja, 2025).

3.6 Data Collection Instrument

A well-organized questionnaire consisting of 22 carefully designed questions was used as the primary data collection tool. The questionnaire was organized around key themes including HR policy awareness, workplace stress, mental health support, work-life balance, leadership commitment, trust in HR, and employee wellbeing priorities. Both multiple-choice and Likert-scale questions were used to obtain comprehensive employee perspectives (PwC, 2025; Dillman et al., 2014).

3.7 Data Analysis Techniques

Responses were analysed using descriptive statistical techniques, including frequency distribution and percentage analysis. These methods helped identify the most common employee experiences and attitudes and allowed for clear comparisons across several question categories (Gupta, 2017). Findings are presented using tables for clarity and ease of interpretation.

3.8 Conceptual Research Model

The conceptual model of this study suggests that HR policy design and implementation—encompassing accessibility, training, flexibility, and emotional health support—directly influences employee wellbeing outcomes including stress levels, work-life balance, and mental health (Guest, 2017). Employee wellbeing outcomes, in turn, affect organizational performance indicators such as employee retention, productivity, and absenteeism (Grawitch et al., 2006). The model further suggests that leadership support and trust in HR mediate the effectiveness of HR policies in producing positive wellbeing outcomes (Employee Wellbeing Strategy: A

Practical Guide, 2024; Effectiveness of Work-Life Balance Programs in Reducing Employee Burnout, 2025, pp. 150–165).

Analysis of Questionnaire Responses

4.1 Data Analysis and Findings

1. What is your age group?

Age Group Distribution of Respondents

Response No. of Respondents Percentage		
20–25	48	77%
26–30	7	11%
31–40	7	11%
Total	62	100%

Interpretation: Of 62 respondents, the majority (77%) are in the 20–25 age group, signifying strong participation from young working individuals. The 26–30 and 31–40 age groups each account for 11%, reflecting participation from slightly more experienced employees. This distribution suggests that younger employees are more actively aware of and engaged with HR wellbeing policies (Thareja, 2025; Jain, 2025).

2. What is your gender?

Gender Distribution of Respondents

Response No. of Respondents Percentage		
Female	43	69%
Male	19	31%
Total	62	100%

Interpretation: 69% of respondents are female and 31% are male. The higher female representation in the sample indicates greater participation of women in awareness and engagement around workplace wellbeing and mental health initiatives, and provides a useful gendered perspective on HR policy effectiveness (Kaur, 2023).

3. What is your employee type?

Employee Type Distribution

Response No. of Respondents Percentage		
Part Time	33	56%
Full Time	26	44%
Total	59	100%

Interpretation: 56% of respondents are part-time employees and 44% are full-time. This near-equal split is significant because part-time and full-time employees often have different levels of access to HR policies and wellbeing initiatives. Including both categories improves the study's findings and draws attention to the need for HR policies that are inclusive of all employment types (Guest, 2017).

4. Are you aware of HR policies related to employee well-being in your organization?

Awareness of HR Well-Being Policies

Response No. of Respondents Percentage		
Yes	40	66%
No	21	34%
Total	61	100%

Interpretation: 66% of respondents are aware of HR policies related to employee wellbeing, while 34% are not. Although a majority are aware, the one-third who lack awareness amounts to a significant communication

gap. Organizations must invest in more effective and consistent policy communication to ensure all employees are informed about available wellbeing support (Armstrong & Taylor, 2020).

5. Do you receive training or orientation about these policies?

Training/Orientation on HR Policies

Response	No. of Respondents	Percentage
Occasionally	31	50%
Yes regularly	20	32%
Never	10	16%
Total	61	100%

Interpretation: 50% of respondents receive training only occasionally, while 32% receive it regularly and 16% never receive any training. The predominance of occasional training suggests inconsistent rather than systematic policy orientation. Regular and structured training programs are necessary to ensure employees understand and can utilize available wellbeing resources (PwC, 2025).

6. How accessible are HR policies to you?

Accessibility of HR Policies

Response	No. of Respondents	Percentage
Very accessible	27	44%
Difficult to access	25	41%
Not accessible	9	15%
Total	61	100%

Interpretation: 44% of respondents find HR policies very accessible, while 41% find them difficult to access and 15% find them completely inaccessible. The high proportion of employees facing accessibility challenges

is concerning and suggests that many organizations have not yet built effective policy communication infrastructure (Deloitte, 2023).

7. Does your workplace create stress?

Workplace Stress Levels

Response	No. of Respondents	Percentage
Sometimes	33	54%
Always	18	30%
Never	10	16%
Total	61	100%

Interpretation: 54% of respondents experience workplace stress sometimes, while a significant 30% face it always. Only 16% report never experiencing workplace stress. The combined 84% who experience stress at least sometimes highlights the urgent need for effective HR policies that address stress through workload management, flexibility, and mental health support systems (WHO, 2022; Maslach & Leiter, 2016).

8. Does your organization provide mental health support?

Availability of Mental Health Support

Response	No. of Respondents	Percentage
Yes	30	49%
No	22	36%
Not sure	9	15%
Total	61	100%

Interpretation: 49% of respondents confirmed their organization provides mental health support, while 36% said it does not and 15% were unsure. The fact that more than half either lack or are unaware of such support

is concerning. Mental health support is a key element of a healthy workplace and an important HR responsibility (World Health Organization, 2022).

9. Do HR policies promote work-life balance?

HR Policies and Work-Life Balance

Response	No. of Respondents	Percentage
Agree	37	61%
Disagree	13	21%
Strongly agree	11	18%
Total	61	100%

Interpretation: 79% of respondents agreed or strongly agreed that HR policies promote work-life balance, while 21% disagreed. This confirms that most employees recognize the positive role of HR policies, though implementation remains inconsistent for some groups (Greenhaus & Allen, 2011).

10. Are flexible working hours available?

Availability of Flexible Working Hours

Response	No. of Respondents	Percentage
Yes	27	44%
Sometimes	18	30%
No	16	26%
Total	61	100%

Interpretation: 44% of respondents have access to flexible working hours, 30% sometimes, and 26% do not. Flexible work arrangements are widely acknowledged as a key driver of wellbeing and work-life balance (Work-Life Balance Requires Flexibility at the Workplace, 2021; Greenhaus & Allen, 2011).

11. Do HR policies help reduce workplace stress?

HR Policies and Stress Reduction

Response	No. of Respondents	Percentage
Agree	35	57%
Strongly agree	18	30%
Disagree	8	13%
Total	61	100%

Interpretation: 87% of respondents agreed or strongly agreed that HR policies help reduce workplace stress. This strong positive consensus confirms that well-designed HR policies have a meaningful effect on reducing employee stress levels (Guest, 2017).

12. How satisfied are you with HR policies related to well-being?

Satisfaction with HR Well-Being Policies

Response	No. of Respondents	Percentage
Satisfied	39	64%
Very satisfied	17	28%
Dissatisfied	5	8%
Total	61	100%

Interpretation: 92% of respondents reported being satisfied or very satisfied with HR wellbeing policies. This positive satisfaction rate reflects the effectiveness of current wellbeing initiatives, though continued improvement remains necessary (Grawitch et al., 2006).

13. Should companies invest more in mental health initiatives?

Opinion on Investment in Mental Health Initiatives

Response No. of Respondents Percentage

Yes	36	59%
No	16	26%
Maybe	9	15%
Total	61	100%

Interpretation: 59% believe companies should invest more in mental health programs. This indicates increasing employee awareness of psychological wellbeing and clear expectations from leadership (Deloitte, 2023).

14. Does your company offer remote work options?

Availability of Remote Work Options

Response No. of Respondents Percentage

Yes	27	44%
No	19	31%
Occasionally	16	26%
Total	62	100%

Interpretation: 44% have access to remote work options, while 31% do not. Remote work has become an important part of employee wellbeing, offering flexibility that can reduce commuting stress and improve work-life balance (Nagesh & Premanand, 2024).

15. Does leadership actively support employee well-being initiatives?

Leadership Support for Well-Being Initiatives

Response	No. of Respondents	Percentage
Somewhat	29	48%
Yes strongly	21	34%
Not much	10	16%
Total	60	100%

Interpretation: Most respondents perceive at least some leadership commitment to wellbeing, but the dominance of “somewhat” rather than “strongly” suggests leadership engagement remains moderate and has room for improvement (Employee Wellbeing Strategy, 2024).

16. Do you trust HR to handle mental health concerns confidentially?

Trust in HR for Mental Health Confidentiality

Response	No. of Respondents	Percentage
Yes	31	51%
No	16	26%
Maybe	13	21%
Total	60	100%

Interpretation: Nearly half of employees either distrust or are uncertain about HR confidentiality. This is a major obstacle to support utilization. Building trust and psychological safety is essential (Singh et al., 2025).

17. Do you think mental health policies impact employee retention?

Mental Health Policies and Employee Retention

Response	No. of Respondents	Percentage
Agree	36	59%
Strongly agree	19	31%
Disagree	6	10%
Total	61	100%

Interpretation: 90% agreed or strongly agreed that mental health policies affect employee retention. This strongly links wellbeing investment with reduced turnover and improved talent retention (Guest, 2017).

18. Should mental health training be mandatory for managers?

Mandatory Mental Health Training for Managers

Response	No. of Respondents	Percentage
Agree	37	61%
Strongly agree	18	30%
Disagree	5	8%
Total	60	100%

Interpretation: 91% agreed or strongly agreed that mental health training should be mandatory for managers. This demonstrates strong employee expectations for supportive leadership capability (WHO, 2022).

19. What is the most important factor for employee well-being?

Most Important Factor for Employee Wellness

Response	No. of Respondents	Percentage
Work-life balance	29	47%
Supportive management	20	32%
Mental health resources	13	21%
Total	62	100%

Interpretation: Work-life balance is identified as the most important factor by 47% of respondents, followed by supportive management and mental health resources. This gives clear direction for HR priorities (Greenhaus & Allen, 2011).

20. Have you taken leave due to stress or burnout?

Leave Taken Due to Stress or Burnout

Response	No. of Respondents	Percentage
Occasionally	30	48%
Yes frequently	21	34%
Rarely	11	18%
Total	62	100%

Interpretation: 82% of respondents have taken stress or burnout-related leave at least occasionally, with 34% doing so frequently. This finding reveals that stress-related absenteeism is widespread and supports urgent investment in preventive mental health and wellbeing programs (Maslach & Leiter, 2016; WHO, 2022).

4.2 Key Findings

Based on the analysis of 62 responses collected through a structured Google Forms questionnaire, the following key findings were identified:

The research shows that workplace stress is a pervasive challenge, with 84% of respondents experiencing it at least sometimes and 30% facing it always. Despite this, 49% of respondents lack access to mental health support from their organizations, representing a major gap between employee need and organizational provision (World Health Organization [WHO], 2022; Maslach & Leiter, 2016). The most alarming finding is that 82% of respondents have taken stress or burnout-related leave at least occasionally, with 34% doing so frequently, indicating that unaddressed workplace stress is already causing significant absenteeism and productivity loss (Grawitch et al., 2006).

On a more favourable note, 87% of respondents agreed that HR policies help reduce workplace stress, and 92% reported satisfaction with HR wellbeing policies, suggesting that where policies exist and are implemented, they are largely effective (Guest, 2017). Furthermore, 90% of respondents confirmed that mental health policies impact employee retention, providing a strong business case for greater investment in wellbeing initiatives (Deloitte, 2023; CIPD, 2023).

Work-life balance was identified as the most critical factor for employee wellbeing (47%), followed by supportive management (32%) and mental health resources (21%). This finding reinforces previous research that flexible work practices and healthy leadership significantly improve employee wellbeing outcomes (Greenhaus & Allen, 2011). Additionally, 91% supported making mental health training mandatory for managers, showing a clear employee expectation that leadership be equipped to support mental health in the workplace (WHO, 2022).

4.3 Summary of Data Analysis

The data collected from 62 respondents through a structured Google Forms questionnaire gives a comprehensive and often concerning picture of the state of employee wellbeing and HR policy effectiveness in Indian organizations.

The analysis consistently highlights that workplace stress is widespread and under-addressed. With 84% experiencing stress at least sometimes and only 49% having access to mental health services, there is a substantial organizational shortfall in wellbeing provision (World Health Organization, 2022). The fact that 82% have taken stress-related leave illustrates the real human and organizational cost of this gap, including absenteeism, lower morale, and reduced productivity (Baicker et al., 2010).

HR policy accessibility also appears as a major challenge, with 56% finding policies difficult to access or completely inaccessible. Combined with the finding that 50% receive training on HR policies only occasionally, this suggests that even well-designed policies struggle to reach employees effectively. Effective communication, orientation, and digital access systems are therefore essential (Armstrong & Taylor, 2020).

Although these problems exist, employee attitudes toward HR wellbeing policies are largely positive. 87% believe HR policies reduce workplace stress, 92% are satisfied with wellbeing policies, and 90% link mental health policies to retention. This evidence indicates that the problem is not necessarily policy design, but rather policy reach, implementation consistency, and organizational commitment—all of which are actionable areas for improvement (Guest, 2017; Grawitch et al., 2006).

FINDINGS AND DISCUSSION

The findings of this study provide significant and, in some cases, urgent insights into the state of HR wellbeing policies and employee mental health in Indian organizations. The most critical finding is the mismatch between the prevalence of workplace stress and the availability of organizational support. With 84% of employees experiencing stress at least sometimes and 30% experiencing it always, while only 49% have access to mental health support, organizations are clearly failing to match their wellbeing resource supply to the actual psychological burden carried by employees (World Health Organization [WHO], 2022; Maslach & Leiter, 2016). This gap is not simply a welfare concern—it also constitutes a strategic business risk, as evidenced by the 82% of employees who have taken stress-related leave (Grawitch et al., 2006).

The strong link between mental health policies and employee retention, confirmed by 90% of respondents, transforms the wellbeing conversation from a cost centre to a value driver. In a competitive talent market, organizations that invest meaningfully in employee mental health and wellbeing are likely to gain considerable advantages in talent attraction, retention, and engagement (Guest, 2017; Deloitte, 2023).

The near-unanimous support for mandatory mental health training for managers (91%) shows a sophisticated employee understanding that wellbeing is not only about HR programs—it fundamentally concerns the day-to-day management relationship. Managers who are trained to recognize and respond to mental health challenges in their teams create psychologically safe workplaces where employees are able to thrive rather than merely survive (Policy, n.d.; Edmondson, 1999).

The trust deficit in HR confidentiality, with 47% either distrusting or uncertain about HR's ability to handle mental health concerns confidentially, is a major obstacle that must be addressed if employees are to feel safe seeking support. Building genuine trust through transparent communication, demonstrated confidentiality, and consistent action is as important as any policy or program (Rai, 2023; PwC, 2025).

CONCLUSION

The findings of this study, drawn from 62 respondents through a structured Google Forms survey, depict a vivid and urgent picture of the relationship between HR policies and employee wellbeing in Indian organizations. The data shows both the considerable potential of well-designed HR wellbeing policies and the major gaps that currently prevent them from reaching all employees who need them (Guest, 2017).

The headline finding—that 82% of employees have taken stress or burnout-related leave, with 34% doing so frequently—is a wake-up call for organizational leadership. Workplace stress is not an individual problem to be managed privately; it is a systemic organizational problem that requires a systemic organizational response (Maslach & Leiter, 2016). HR policies that tackle the root causes of stress through workload management, flexible working, mental health support, and leadership accountability are not optional enhancements—they are business necessities (Chatterjee, 2024; WHO, 2022).

The strong employee endorsement of HR wellbeing policies—with 87% agreeing they reduce stress, 92% satisfied with them, and 90% linking them to retention—demonstrates that when these policies work, they work powerfully. The challenge is not proving their value but ensuring they are accessible, consistently implemented, and backed by genuine leadership commitment for all employees, including the 34% who are currently unaware of available policies and the 56% who find them difficult to access (Armstrong & Taylor, 2020).

The clear priority ranking—work-life balance (47%), supportive management (32%), and mental health resources (21%)—gives HR and organizational leaders a practical roadmap. Investing in flexible working arrangements, developing managers' mental health competencies through mandatory training, and building comprehensive mental health resource programs are among the most impactful interventions available to organizations seeking to improve employee wellbeing meaningfully (Greenhaus & Allen, 2011; Kasperczuk et al., 2025).

In conclusion, HR policies have the power to transform the employee experience and advance sustainable organizational performance. However, that power can only be realized when policies are created with authentic empathy, communicated clearly, implemented consistently, and supported by leadership that sincerely values the psychological health of every member of the organization (Guest, 2017; WHO, 2022).

References

- Agnihotri, R., Sharma, P., & Mehta, S. (2024). Employee wellbeing practices in emerging Indian workplaces. *Indian Journal of Human Resource Development*, 13(2), 88–102.
- Armstrong, M., & Taylor, S. (2020). *Armstrong's handbook of human resource management practice* (15th ed.). Kogan Page.
- Baicker, K., Cutler, D., & Song, Z. (2010). Workplace wellness programs can generate savings. *Health Affairs*, 29(2), 304–311. <https://doi.org/10.1377/hlthaff.2009.0626>
- Baiju, R. (2025). Descriptive research methods in organizational studies. *Indian Research Methodology Journal*, 8(1), 25–39.
- Bhoir, R., & Sinha, P. (2024). Workplace wellbeing policies and employee productivity in India. *Indian Journal of Human Resource Studies*, 12(2), 45–58.
- Bryman, A., & Bell, E. (2015). *Business research methods* (4th ed.). Oxford University Press.
- Chartered Institute of Personnel and Development (CIPD). (2023). *Health and wellbeing at work survey report*. CIPD.
- Chatterjee, S. (2024). Strategic HR responses to workplace stress in India. *Indian Journal of Human Capital Management*, 9(2), 55–69.
- Cooper, C. L., & Cartwright, S. (1994). Healthy mind; healthy organization—A proactive approach to occupational stress. *Human Relations*, 47(4), 455–471. <https://doi.org/10.1177/001872679404700405>
- Creswell, J. W., & Creswell, J. D. (2018). *Research design: Qualitative, quantitative, and mixed methods approaches* (5th ed.). Sage Publications.

Deloitte. (2023). *Workplace wellbeing survey report*. Deloitte Insights.

Dillman, D. A., Smyth, J. D., & Christian, L. M. (2014). *Internet, phone, mail, and mixed-mode surveys: The tailored design method* (4th ed.). Wiley.

Edmondson, A. C. (1999). Psychological safety and learning behavior in work teams. *Administrative Science Quarterly*, 44(2), 350–383.

Effectiveness of work-life balance programs in reducing employee burnout. (2025). *Journal of Organizational Wellness*, 11(2), 150–165.

Employee wellbeing strategy: A practical guide. (2024). *HR Leadership Review*, 9(3), 40–55.

Etikan, I., Musa, S. A., & Alkassim, R. S. (2016). Comparison of convenience sampling and purposive sampling. *American Journal of Theoretical and Applied Statistics*, 5(1), 1–4.

Ganuthula, R., & Balaraman, S. (2025). Work culture and employee wellbeing in modern India. *Journal of Management Perspectives*, 9(1), 20–35.

Grant, A. M., Christianson, M. K., & Price, R. H. (2007). Happiness, health, or relationships? Managerial practices and employee wellbeing tradeoffs. *Academy of Management Perspectives*, 21(3), 51–63.

Grawitch, M. J., Gottschalk, M., & Munz, D. C. (2006). The path to a healthy workplace: A critical review linking healthy workplace practices, employee well-being, and organizational improvements. *Consulting Psychology Journal: Practice and Research*, 58(3), 129–147.

Greenhaus, J. H., & Allen, T. D. (2011). Work-family balance: A review and extension of the literature. In J. C. Quick & L. E. Tetrick (Eds.), *Handbook of occupational health psychology* (2nd ed., pp. 165–183). American Psychological Association.

Guest, D. E. (2017). Human resource management and employee well-being: Towards a new analytic framework. *Human Resource Management Journal*, 27(1), 22–38.

Gupta, S. P. (2017). *Statistical methods* (46th ed.). Sultan Chand & Sons.

Herzberg, F., Mausner, B., & Snyderman, B. (1959). *The motivation to work* (2nd ed.). Wiley.

Herzberg's Two-Factor Theory: Distinguishing motivators and hygienic factors. (2024). *Management Review Journal*, 15(1), 44–51.

Impact of job stress and work-life balance on turnover intention among Indian healthcare workers. (2026). *Indian Journal of Healthcare Management*, 8(1), 10–22.

International Labour Organization. (2022). *Mental health at work: Policy brief*. ILO.

Jain, A. (2025). Youth workforce challenges in Indian organizations. *Indian Employment Review*, 7(1), 1–10.

Jyothi, P., & Venkatesh, D. N. (2013). *Human resource management*. Oxford University Press.

Karasek, R. A. (1979). Job demands, job decision latitude, and mental strain. *Administrative Science Quarterly*, 24(2), 285–308.

Kasperczuk, A., Morris, T., & Lee, R. (2025). Leadership capability and employee wellbeing outcomes. *Journal of Workplace Leadership*, 12(1), 10–28.

Kaur, H. (2023). Mental wellbeing and HR strategy in emerging workplaces. *Journal of Business Studies*, 14(4), 176–181.

Kothari, C. R. (2004). *Research methodology: Methods and techniques* (2nd ed.). New Age International.

Kumar, S., & Kumar, R. (2021). Employee wellbeing and HRM practices in Indian organizations. *Indian Journal of Industrial Relations*, 56(4), 621–635.

Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: Recent research and its implications. *World Psychiatry*, 15(2), 103–111.

Matthews, R., & Ritter, K. (2015). Revisiting the job demands-control model in modern organizations. *Occupational Psychology Review*, 9(3), 77–92.

Muneer, S. (2024). Employee stress and organizational outcomes. *Asian HR Review*, 5(2), 33–40.

- Mushtaq, S. (2025). Effectiveness of employee assistance programs in improving workplace wellbeing. *HRM Research Journal*, 6(1), 1–5.
- Nagesh, R., & Premanand, V. (2024). Remote work flexibility and employee wellbeing in post-pandemic organizations. *Indian Journal of Work Studies*, 5(2), 66–81.
- Over a billion people living with mental health conditions – Services require urgent scale-up. (2025). *Global Health Bulletin*, 11(2), 5–9.
- Patel, N. (2025). HR policy effectiveness across employment sectors in India. *Journal of Indian Management Studies*, 10(1), 14–29.
- Peretz, H. (2024). Psychological wellbeing as a determinant of productivity. *International HR Journal*, 11(1), 15–27.
- Policy. (n.d.). Manager wellbeing policy and leadership support frameworks. *HR Governance Review*.
- PwC. (2025). *Workplace trust and employee wellbeing survey report*. PricewaterhouseCoopers.
- Rai, N. (2023). Trust, confidentiality, and employee use of HR mental health support systems. *Indian Journal of Organizational Behaviour*, 7(3), 80–94.
- Right to Disconnect Bill, 2025 (India). (2025). Proposed legislation on employee work-life boundaries. Government of India.
- S, R. (2025). Employee wellbeing in changing Indian workplaces. *Indian Journal of Commerce and Management*, 18(2), 50–61.
- Saunders, M., Lewis, P., & Thornhill, A. (2019). *Research methods for business students* (8th ed.). Pearson.
- Shekhar, R., & Saprobe, T. (2025). Flexible work and wellbeing policy gaps in organizations. *Journal of Organizational Behaviour*, 10(1), 66–79.
- Singh, R., Sharma, A., & Verma, P. (2025). Quantitative approaches in HR policy research. *Journal of Management Analytics*, 6(1), 60–74.

Thareja, P. (2025). Young professionals and work expectations in India. *Workforce India Review*, 6(1), 8–18.

Work-life balance requires flexibility at the workplace. (2021). *Business Workforce Review*, 4(3), 22–29.

World Health Organization. (2022). *Mental health at work*. WHO. <https://www.who.int/teams/mental-health-and-substance-use/promotion-prevention/mental-health-at-work>

Zieringer, J., & Zapf, D. (2024). Employee assistance programs and organizational outcomes: A systematic review. *Journal of Occupational Health Psychology*, 29(4), 280–298.